Adult advanced NSCLC · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. NCCP 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

NCCP 00243 · VERSION 7

Crizotinib monotherapy

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Vision changes, sometimes severe visual loss

☐ Swelling, tiredness and nerve symptoms

☐ Liver injury, which can be serious or fatal

☐ Slow heart rate, abnormal heart rhythm and heart failure

☐ Skin rash and sensitivity to sunlight

☐ Nausea, vomiting, diarrhoea or constipation; appetite change

☐ Low blood counts and infection

☐ Lung inflammation (ILD/pneumonitis), which can be serious or fatal

☐ Kidney injury; rarely bowel perforation

☐ Potential harm to a pregnancy; fertility may be affected

Discuss the selected ALK/ROS1 indication and treatment line, individual aim and uncertainty, alternatives including not proceeding, material and lasting risks, interactions, fertility/pregnancy/feeding, daily treatment burden, monitoring, and your right to reconsider. This discussion aid is not consent or treatment authorisation.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: confirm the tumour test, blood count, kidney/liver tests, chest imaging, symptoms and all medicines; ECG and electrolytes near the first dose. Liver tests are planned weekly for the first 2 months using current product information, then monthly; NCCP says every 2 weeks initially, so confirm the local plan. Blood count and kidney tests are monthly. ECG, electrolytes, pulse and blood pressure need a planned schedule and earlier review if unwell. NCCP lists monthly chest X-ray; other disease imaging follows your consultant plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.